Pemetrexed Monotherapy

Which treatment this guide covers This guide covers pemetrexed on its own for maintenance or second-line treatment of selected non-squamous non-small-cell lung cancer. If pembrolizumab or another immunotherapy is also prescribed, use the matching combination regimen and its additional consent, monitoring and urgent advice. Pemetrexed with pembrolizumab maintenance is a different treatment plan.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pemetrexed alone · treatment day

Active medicines: pemetrexed

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally once daily	Treatment day (Day 1) NCIS V8 low-risk reference. Review local policy, QT risk and individual suitability; antiemetic class guidance permits a 5-HT3 antagonist or corticosteroid.
Metoclopramide	10 mg orally three times daily as required	Subsequent days, as prescribed NCIS low-risk row; prescriber/pharmacy must specify appropriate duration and assess contraindications, renal adjustment and interacting medicines.
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and21 days after final dose Continue across a platinum-to-pemetrexed phase change.
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed, then every three cycles Later injections may be given on pemetrexed day; reconcile the existing schedule at phase changes.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed One reconciled steroid prescription. Do not add the previous platinum antiemetic steroid course or duplicate this for antiemesis.

Additional phase-specific support

- Standalone pemetrexed is low emetogenic risk; review and replace the prior platinum support plan when that phase ends.
- Daily folate and B12 every three cycles continue while pemetrexed continues.
- Rash steroid is one reconciled prescription; no second antiemetic dexamethasone course is added.
- Review renal function and NSAID/aspirin interactions against the current SmPC.
- Routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or antimicrobial prophylaxis is not mandated by NCCP00222; individualise after clinical risk or toxicity review.
- Pembrolizumab/pemetrexed maintenance requires the combination regimen and separate immune safety plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00222 v5 · p1–4 · NCCP NCIS Medical Oncology antiemetics V8, Table 1 low-risk row, page 4

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required